

EBOLA VIRUS DISEASE

ORTHOEBOLAVIRUSES

1

CALL CLINICALL

2

PERFORM PCRA

3

**PREVENT
EXPOSURE**

4

**VENTILATE
& CLEAN**

WHEN TO SUSPECT EBOLA

EXPOSURE RISK

- Travel to affected region within last 21 days
- Contact with suspected / confirmed case

EARLY SYMPTOMS

- Fever, chills
- Headache, sore throat
- Weakness, fatigue
- Muscle and joint pain

PROGRESSIVE/SEVERE SYMPTOMS

- Vomiting and diarrhea
- Rash or red eyes
- Jaundice
- Chest or abdominal pain
- Severe weight loss
- Delirium or shock
- Bruising or bleeding



HIGHER TRANSMISSION RISK

Risk increases significantly with:

- Vomiting
- Diarrhea
- Bleeding / hemorrhage



STRICT PPE

- Droplet + Contact
- Not airborne in routine care
- Fit-tested N95 respirator for high risk procedures
- Eye protection
- Gloves
- Fluid impermeable gown / coverall



CLEAN → DISINFECT

- Use approved disinfectant on:
 - Patient compartment
 - Stretcher & equipment
 - All high-touch surfaces
- Remove visible contamination first
- Wipe (no spraying)

PREPARE TRANSPORT

- Remove/cover equipment
- Use barriers where possible
- Keep contamination contained
- Ventilate ambulance for **20min** if AGMPs used
- Avoid re-entry ventilation



DURING CARE

- Avoid contact with body fluids
- Mask patient (if tolerated)
- Limit to essential crew
- Avoid unnecessary AGMPs

WASTE (CATEGORY A)

- Treat all waste as high risk
- Double-bag
- Do not compact
- Biohazard container
- Follow protocol as per local hospital



Patients in quarantine without symptoms should receive timely care using routine precautions. Contact CliniCall and perform PCRA, escalate precautions as needed. IDCT teams have advanced IPAC training and specialized PPE/equipment (e.g. double gloving, apron use) for high-risk patient transport.